Inbrev

Hack Apertus 2026 · Track 2B · Team Apertool

# Run locally, the practice inbox never leaves the practice.

Inbrev sorts the inbox of a Swiss GP practice by handling path and deadline, with Apertus, built to run on a box in the practice. Every field it extracts quotes its source.

▶ Watch the 2-min demo

Code on GitHub

solo builder · fictional data only

THE PROBLEM

# Before the first patient, there is the inbox.

Lab reports, refills, insurer forms and faxes in three languages land in one queue. A practice assistant (MPA) reads them one by one.

**9.5 h / week**

of administration for Swiss GPs, up from 8.6 h in 2020

Workforce Study, *Praxis* 2026 · n = 1,776

**54 min / day**

on requests from authorities and insurers alone

gfs.bern for FMH, 2025 · N = 1,532

**Art. 321**

Professional secrecy binds doctors **and their assistants**

Swiss Criminal Code · FADP Art. 5

HOW IT WORKS · PURPOSEFUL USE OF AI

# Sorted by who handles it and by when.

01 · READ

## Mail, forms, faxes

E-mail, IMAP, web forms, fax via OCR. German, French, Italian, English.

02 · ROUTE

## 9 handling paths

Plus the administrative deadline and who takes it.

03 · PROVE

## Quote or drop

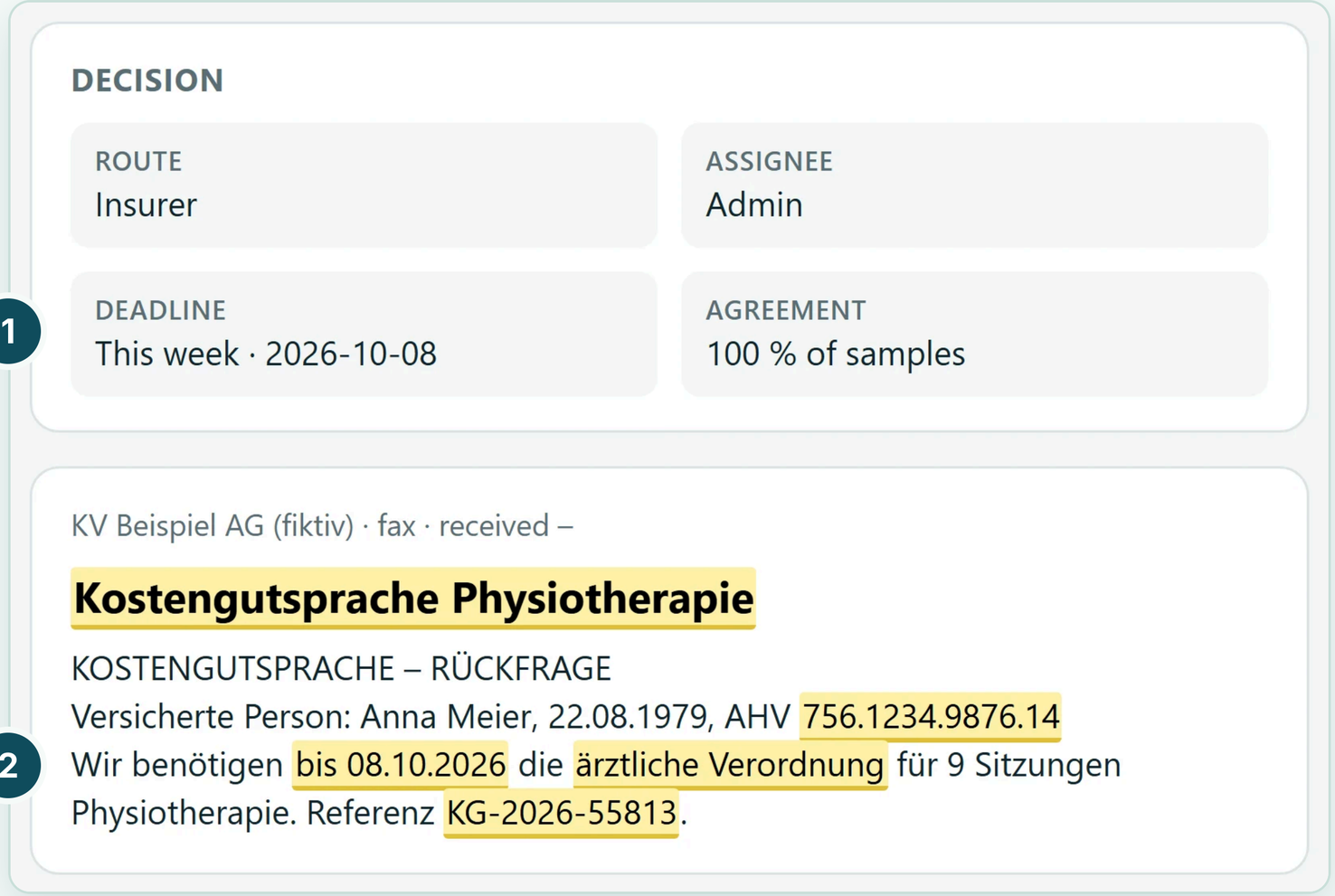
Every field quotes its source. No quote in the original, no field.

**Clinical content** Possible symptoms leave automation and go to a person. Not judged, not ranked.

Code checks what code can check: AHV numbers, dates, quotes.

DEMO

# One fax, sorted. Every value shows where it came from.




**1** **Route and deadline**

Insurer, admin, by 8 Oct. The date comes from the quote, the deadline from code.

**2** **Quote or drop**

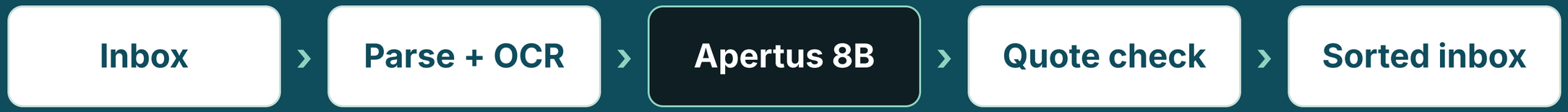
Each highlighted value is verbatim in the fax. No quote, no value.

Running app, stored Apertus 8B result, fictional fax. Possible symptoms would go to a person instead.

ARCHITECTURE · SOVEREIGN DEPLOYABILITY

# Pull the cable. Inbrev keeps sorting.

INSIDE THE PRACTICE · NO OUTBOUND CONNECTION




| | |
|---|---|
| **On-prem** | mini PC in the practice |
| **Air-gapped** | mail gateway hands messages in |
| **Swiss cloud** | option for single practices |

```
Offline run, measured
egress dns / tcp / https: FAILED
Q4 weights, laptop CPU: 192 s / message
```

EVALUATION · TECHNICAL RIGOUR

# No clinical message ran by itself.

180 synthetic messages, labels not reviewed by a person.

| Setup | Routes right | Clinical out | Runs by itself |
|---|---|---|---|
| Keyword rules + lexical net | 0.79 | 7/16 | 88 % |
| Apertus 8B, one answer, no checks | 0.87 | 9/16 | 95 % |
| Inbrev with Apertus 70B | 0.96 | 15/16 | 82 % |
| **Inbrev 8B + symptom pass** | **0.94** | **16/16** | **78 %** |

Paired on 180: 8B beats the rules 32 to 5 ($p = 7 \cdot 10^{-6}$). Details and all rows: report §5.

**16/16**

clinical messages out of automation
95 % CI 0.81–1.00 (Wilson)

- Labels fixed before any text; Apertus never writes or labels test data.
- Two open models (not people) relabelled 367 messages blind: κ 0.93 route, 0.97–0.98 clinical. Human review pending.

PURPOSEFUL USE OF AI · MODEL CHOICE

# Why Apertus, and what it costs.

## Measured, same pipeline

- ✕ Qwen3.5-9B, Ministral 8B: 177/180 routes
- ✕ Apertus 8B: 170/180 · 70B: 172/180
- ✕ Fresh hold-out: 5 wrong routes run by themselves (Qwen 0); 70B second opinion: 3

## Why Apertus anyway

- ✓ Only openly available training data, opt-outs applied, personal data filtered
- ✓ Published data catalogue and EU training-content summary
- ✓ Local run and quote checks work with any model: no argument

**A trade-off, not a free choice.** Sources and hold-out details: report §5.

VALUE, COST & SCALABILITY

# Swiss francs per 1,000 messages, on hardware the practice owns.

ON-PREM MINI PC

**≈ CHF 12**

at 60 messages a day, box and power

SWISS CLOUD VM

**CHF 2.39**

billed while sorting

SWISS TOKEN API

**CHF 2.59**

reference, Apertus 70B list price

**≈ 32 min**

MPA time saved a day ASSUMPTION

60 messages × 78 % automatic × 45 s saved, minus rework of misroutes

Prices excl. VAT, 4 Oct 2026. Model, sources and break-even: report §5.

INTENDED PURPOSE · IMPLEMENTATION FEASIBILITY

# Administrative by default. Clinical mode is research only.

### Inbrev does

- ✓ Sort by path and deadline
- ✓ Extract fields, each with its quote
- ✓ Say “you decide” when unsure

### Inbrev never does

- ✗ Rank by clinical severity
- ✗ Assess symptoms or suggest care
- ✗ Send patient data out (local mode)

Clinical research mode: off by default, not validated, not certified (MDCG 2019-11, MepV Art. 15).

BUSINESS MODEL & ROADMAP

# A module for the software practices already use.

| | |
|---|---|
| **User** | The MPA, first thing in the morning |
| **Buyer** | Practice owner or doctors' network |
| **Price** | CHF 150–250 / month HYPOTHESIS |
| **Channel** | Module in HIN mail or practice software |

**OCT 2026**
Evaluation, offline proof, cost

**NOV – DEC 2026**
5–10 interviews with MPAs and owners

**Q1 2027**
Pilot in one group practice

**Q2 2027**
Classification review before first sale

Inbrev

**Team Apertool** · Niklas Wichter, solo builder

OUR ASK

# Pilot Inbrev in one Swiss group practice.

On its own box, from Q1 2027. Your inbox stays in your practice.

Contact: hello@apertool.ch

inbrev.apertool.ch

**Demo** ▶ 2-min video

**Code** github.com/NWichter/Hack_Apertus_2B_Inbrev